

14 DAY WAIT – URGENT REFERRAL

ROYAL STOKE UNIVERSITY HOSPITAL

ALL REFERRALS TO BE SENT VIA RAS (REFERRAL ASSESSMENT SERVICE)
YOU ARE NO LONGER REQUIRED TO EMAIL/FAX UBRN

Gynaecology

PATIENT DETAILS			GP DETAILS			
Name:	Full Name		Name:	Organisation Name		
	Home Full Address (single line)			Organisation Full Address (single line)		
Address:			Phone No:	Organisation Telephone Number		
NHS Number:	NHS Number		Fax No:	Organisation Fax Number		
Hospital number:	Hospital Number		Name of referrer:	Free Text Prompt		
Date of Birth:	Date of Birth		Decision to refer date:	Short date letter merged		
Interpreter/Sign Language required:	☐ Yes ☐ No		Language:			
Contact No (next 48 hrs):	Home:	Patient Home Telephone	Work:	Patient Work Telephone	Mobile:	Patient Mobile Telephone

- ☐ Patient has been informed that this is an urgent referral for suspected cancer
☐ The patient is available and willing to attend hospital for tests/appointment within 14 days
☐ The patient has been given the 2ww Patient Information Leaflet

By selecting this pathway you are referring the patient for a pelvic USS

REASON FOR REFERRAL (please indicate as appropriate)

CERVIX:

- ☐ Lesions suspicious of cancer of cervix on speculum examination
☐ Post-coital bleeding (PCB) age over 35 years that persist for more than 4 weeks (include date and results of last cervical screening; ensure infection is excluded)

INVESTIGATIONS:-

SMEAR	
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For persistent unexplained Post-coital bleeding (PCB) in a woman under 35 years of age consider an urgent referral to General Gynaecology via Choose and Book

VULVA/VAGINA:

- ☐ Unexplained vulval lump, ulceration or bleeding
☐ Unexplained palpable mass in or at the entrance to the vagina

OVARY:

- ☐ Palpable abdominal/pelvic mass or ascites (new finding do not await scan)
☐ CA125 >35 IU/ml and USS OR symptoms suggestive of ovarian cancer (as per NICE guidance)

INVESTIGATION RESULTS:-

CA125:	Single Code Entry: CA-125 arbitrary concentration in serum...
USS:	

Please perform CA125 test in parallel with referral (plus LDH, BHCG, AFP if the patient is under 40 years of age)

UTERUS:

Name: Full Name NHS No: NHS Number

☐ Post-menopausal bleeding or blood stained discharge ☐ Unexplained or prolonged vaginal bleeding persisting for more than 4 weeks after stopping HRT ☐ Irregular bleeding or postmenopausal bleeding in patients taking Tamoxifen <i>For heavy peri-menopausal bleeding over 45 despite medical treatment consider an <u>urgent</u> referral to General Gynaecology via choose and book as per NICE guidelines</i>	
If examination is not possible, please state why	
Any additional clinical details - compulsory (i.e. any further relevant comments)	
<u>PATIENT MEDICAL DATA:</u> <u>Comorbidities:</u> See below <u>Any allergies/anticoagulation's:</u> See below <u>BMI:</u> Single Code Entry: Body mass index	
<u>Accessibility Needs:</u> ☐ Wheelchair access ☐ Deaf ☐ Registered blind ☐ Learning Disability ☐ Other disability needing consideration ☐ Accompanied by carer	<u>WHO Performance Status:</u> ☐ 0 Fully active ☐ 1 Able to carry out light work ☐ 2 Up and about greater than 50% of waking time ☐ 3 Confined to bed/chair for greater than 50% ☐ 4 Confined to bed/chair 100%
<u>RISKS:</u> ☐ Vulnerable Adult (detail below if any recording within last 3 years) ☐ No Capacity to Consent Any other known risk:	

NB: Incomplete form would affect patient's pathway and subject to rejection. Please ensure ALL appropriate information is provided before referral.

Reference: NICE Guideline recommendations for Suspected cancer: recognition and referral, July2017 and West Midlands Cancer Alliance Gynaecology 2WW referral form (ratified March 2021)

Problems

Allergies

Medication

Name: Full Name NHS No: NHS Number